Dartmouth Health

Mt. Ascutney Hospital & Health Center

The 340B program at Mt. Ascutney Hospital & Health Center: Fiscal Year 2025

ABOUT OUR HOSPITAL: Mt. Ascutney Hospital & Health Center (MAHHC) is a 35 bed, not-for-profit Critical Access Hospital with an inpatient rehabilitation unit serving central Vermont, and surrounding communities in Vermont and New Hampshire since 1933. MAHHC and its array of outpatient clinics and services in Vermont and New Hampshire serve a diverse population with unique healthcare needs. In fiscal year 2025 MAHHC provided more than 32,000 primary care and pediatric appointments, over 900 inpatient discharges, more than 1,600 surgeries and procedures, and over 9,500 emergency department and Walk-in visits. The MAHHC Emergency Department is staffed 24/7 in order to provide care for critically ill patients. In addition to inpatient and primary care, MAHHC provides a variety of specialty care services including, cardiology, neurology, ophthalmology, outpatient infusion services, podiatry, rehabilitation services, rheumatology, and surgical services. MAHHC is a proud member of the Dartmouth Health system.

As one of the largest employers in the area, MAHHC employs over 523 clinical and administrative staff members.

In its fiscal year 2024 IRS 990 filing, MAHHC reported more than $7.582M in at-cost charitable benefits. These benefits include:

- $5.3M loss on Medicaid
- $283K in Charity Care
- $2.0M in Community Health, Education and Subsidized services.

THE BENEFITS OF THE 340B PROGRAM: The 340B program offsets a significant amount of operating expense and loss, and enables MAHHC to provide several much-needed services to patients and the community, all with a goal of increasing access to healthcare. MAHHC spends approximately $2.2M annually on medications for our patients. The approximate value of MAHHC's 340B benefit in FY25 was $733k, which was used in numerous ways to support patients and their families, as guided by the Community Health Improvement Plan.

MAHHC's FY2024-2025 Community Health Improvement Plan (CHIP) focused on addressing the availability of mental health services, cost of healthcare services, alcohol and substance misuse prevention, socioeconomic issues (including housing, food, childcare), and strengthening families. This was accomplished by supporting the work of Community Health Implementation Plan (CHIP) networks and workgroups as well as collaboration with local and state resources.

Benefits from the 340B program helped to support community benefit programs and initiatives such as:

a. Assisting the uninsured or underinsured through Mt. Ascutney Health Connections by helping patients obtain health insurance, gas cards, medication vouchers, and dental vouchers to help eliminate barriers to healthcare access
b. Providing volunteer services through Volunteers in Action (ViA) including 'Meals on Wheels', 'Rides to Job Access', and 'Rides to Wellness' programs

c. Connecting residents to basic needs, goods, and human service agencies at the Windsor Resource Center including access to mental health and/or substance use counselling, case management from Senior Solutions, tax assistance, family/child playgroups, and free clothing and personal care items
d. Establishing access to walk-in clinic services at Mt. Ascutney Hospital which offers a convenient way for patients to obtain high quality care in urgent situations
e. Collaborating with community partners to host family and child playgroups to increase social connections and support for local families
f. Promoting public awareness and education for older adults on the impact of alcohol/substance misuse, aging in place, and end of life decisions through reading materials and community seminar series
g. Providing alcohol and substance misuse programming through the collaboration of MAHHC Emergency Department and key community health partners to deliver access to Medication Assisted Treatment and recovery coach services.
h. Coordinating food programming and resources within our services area through Veggie Van Go, Windsor Food Shelf, satellite food cupboard at the Windsor Resource Center, and the WSESU summer delivery program for kids

IMPACT IF THE 340B PROGRAM WAS SCALED BACK: The loss of $733K in benefits from the 340B program would have significant impact on our patients and our community. Rural hospitals like MAHHC are a crucial part of the state of Vermont's healthcare delivery system with more than half of the state's hospitals being critical access hospitals. Our hospital provides essential services to patients who would otherwise not have access. The benefits from the 340B program at MAHHC are used as intended in the Public Health Service Act- to stretch scarce resources as far as possible in order to reach more patients and provide more comprehensive services. In addition to providing medication cost savings, if the 340B program were scaled back or eliminated, many of the examples of patient and community support listed above would be negatively impacted.

It is imperative that the 340B program remain in place – as currently structured – for the health and benefit of the patients who need us most.

Internal Review and Oversight: Mt. Ascutney Hospital and Health Center takes its commitment to the 340B Drug Pricing Program very seriously. MAHHC maintains a comprehensive suite of 340B-related policies, procedures, and job aids designed to ensure ongoing compliance and program integrity in accordance with the U.S. Department of Health and Human Services, Health Resources and Services Administration (HRSA) 340B Program requirements and guidance.
These policies support compliance, monitoring, and operational oversight across all major program requirements and include robust controls aimed at preventing diversion and duplicate discounts, the two primary cornerstones of 340B compliance.

MAHHC maintains compliance with the 340B Program by conducting monthly claim audits in accordance with HRSA audit guidance. These audits include Contract Pharmacy claim audits as well as Mixed-Use administration and dispensation claim audits and are performed on a monthly basis in volumes established by organizational policy. Any identified issues are reviewed by the 340B Center of Excellence (CoE) on a monthly or bi-monthly basis, depending on the pharmacy partner, and corrective actions (claim reversals and process improvements) are implemented promptly when claims are determined to be ineligible.

In addition, the MAHHC 340B CoE team meets with applicable MAHHC stakeholders on a bi-monthly basis to review program performance and compliance, as defined by policy. Furthermore, all Dartmouth Health 340B programs, including MAHHC, are reviewed quarterly through the Dartmouth Health 340B Steering Committee. This governance structure ensures that relevant Dartmouth Health C-suite representatives remain informed and engaged regarding 340B oversight and compliance matters.

A key component of MAHHC's 340B compliance framework is the completion of an annual, contracted external audit of the MAHHC 340B Program conducted by our vendor partner, SpendMend. These external audits are structured to mirror the scope and rigor of an HRSA audit and evaluate all required aspects of program compliance and maintenance. Any identified findings or concerns are documented in the final audit report and addressed by MAHHC in a timely and compliant manner.

Internal Expenses: The aggregated payment amount calculated for payments made for all other internal expenses related to administering the 340B program during fiscal year 2025 were derived from a pharmacy shared service allocation from DH, as calculated by DH as MAHHC's share of total system costs. The general management and administration of the 340B program was assumed by DH in FY25.